VANI'S SPEECH & HEARING CLINIC

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VOICE, STUTTERING & DYSPHAGIA EVALUATION REPORT

Patient Name:	Mrs. Sunita M. Deshmukh	**UHID:**	PGC-26-000340
Age / Gender:	42 Yrs / Female	**Date of Test:**	October 09, 2026
Occupation:	School Teacher (Professional Voice User)	**Referred By:**	Dr. A. V. Joshi, MS (ENT)

1. PERCEPTUAL VOICE EVALUATION (CAPE-V & GRBAS)

ASSESSMENT SCALE	PARAMETER EVALUATED	VISUAL ANALOG SCORE / GRADE (0-100 MM / 0-3)	CLINICAL INTERPRETATION
CAPE-V (Consensus Auditory-Perceptual Evaluation of Voice)	Overall Severity	55 / 100 mm (Moderate)	Moderate dysphonia during continuous speech
	Roughness	48 / 100 mm (Moderate)	Audible vocal fold irregularity / raspy tone
	Breathiness	35 / 100 mm (Mild-Moderate)	Audible air escape during sustained vowels
	Strain	60 / 100 mm (Moderate)	Hyperfunctional muscle tension during phonation
	Pitch & Loudness	Pitch: 40 (Slightly Low) | Loudness: 30 (Mild)	Pitch breaks noted during vocal load
	Resonance	Normal (Oral-Nasal Balance)	No hyper/hyponasality detected
GRBAS Scale	G2 (Grade) | R2 (Roughness) | B1 (Breathiness) | A0 (Asthenia) | S2 (Strain)		Moderate voice impairment driven by roughness & strain

2. PHONATORY, ACOUSTIC MEASURES & VOICE HANDICAP INDEX (VHI)

PARAMETER / MEASURE	OBSERVED VALUE	NORMATIVE RANGE	DIAGNOSTIC SIGNIFICANCE
MPT (Maximum Phonation Time)	/a/ = 7.2 sec | /i/ = 8.0 sec | /u/ = 7.5 sec	15.0 – 20.0 seconds	Significantly reduced phonatory breath support
s/z Ratio	/s/ = 14.5 sec | /z/ = 7.2 sec (Ratio = **2.01**)	Ratio < 1.4	Pathological (Indicates glottal insufficiency / lesion)
HNR (Harmonics-to-Noise Ratio)	11.4 dB	> 20.0 dB	Elevated noise component in voice signal
VHI (Voice Handicap Index)	Functional: 18 | Physical: 26 | Emotional: 14 **Total VHI Score = 58 / 120**	0 – 30: Mild 31 – 60: Moderate 61 – 120: Severe	**Moderate Voice Handicap** (High physical handicap in teaching)

3. STUTTERING EVALUATION (SSI-4 & %SS ANALYSIS)

(Evaluated for Pediatric Fluency Profile Patient: Master Rohan K. Shinde, Age 11 Yrs)

SSI-4 SUBSCALE	RAW TEST DATA	CONVERTED TASK SCORE	SEVERITY CLASSIFICATION
Frequency (% Syllables Stuttered)	9.4% SS (Reading + Speaking tasks)	16 Points	**SSI-4 Total Score: 28** **Severity: MODERATE-SEVERE STUTTERING**
Duration	Mean of 3 longest blocks: 2.8 seconds	5 Points	
Physical Concomitants	Facial grimace (3), Eye blink (2), Head nod (2)	7 Points	

4. DYSPHAGIA (SWALLOWING) EVALUATION (FOIS & IDDSI LEVELS)

(Evaluated for Adult Dysphagia Profile Patient: Mr. Vitthal R. Jadhav, Age 71 Yrs, Post-Stroke)

DYSPHAGIA TOOL	ASSESSED LEVEL / STATUS	DIET & FLUID GUIDELINES
FOIS (Functional Oral Intake)	**Level 3: Total oral intake of single consistency**	Requires special preparation and tube compensation backup
IDDSI Fluid Consistency	**IDDSI Level 2: Mildly Thick Liquid**	Nectar-thick consistency; avoids thin water aspiration risk
IDDSI Food Texture	**IDDSI Level 4: Pureed Diet**	Smooth, cohesive, no lumps, no chewing required

DIAGNOSTIC IMPRESSION & RECOMMENDATIONS:
1. **VOICE IMPRESSION**: Moderate Vocal Hyperfunction with Glottal Insufficiency (s/z ratio 2.01, VHI 58). ENT indirect laryngoscopy

recommended to rule out vocal vocal nodules.
2. **STUTTERING IMPRESSION**: Moderate-Severe Fluency Disorder (SSI-4 Score 28, 9.4% SS) with secondary physical concomitants.
3. **DYSPHAGIA IMPRESSION**: Oropharyngeal Dysphagia post-stroke (FOIS Level 3) requiring IDDSI Level 2 (Mildly Thick) fluids and Level 4 (Pureed) diet with chin-tuck swallowing maneuver.

Date: October 09, 2026
Place: Satara

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